



APPENDIX IV

Updated SAP (Clean Version)

Chengdu Kanghong Biotechnology Co., Ltd.

A Multicenter, Double-Masked, Randomized, Dose-Ranging Trial to Evaluate the Efficacy and Safety of Conbercept Intravitreal Injection in Subjects with Neovascular Age-related Macular Degeneration

Clinical Protocol: KHB-1801, Version 3.0, Amendment #1

Statistical Analysis Plan Version 5.0 February 26, 2021

Confidentiality Statement

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February 26, 2021

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Sponsor Approval

Protocol Title: A Multicenter, Double-Masked, Randomized, Dose-Ranging Trial to Evaluate the Efficacy and Safety of Conbercept Intravitreal Injection in Subjects with Neovascular Age-related Macular Degeneration

Protocol Number: KHB-1801, Version 3.0, amendment #1, dated December 11, 2020

Date: 26-FEB-2021

This Statistical Analysis Plan and related documentation was subject to critical review and has been approved by the Sponsor. The following personnel contributed to writing and/or approving this protocol.

Signed: _____

Date: 26-Feb-2021 | 12:49 PM EST

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Protocol Amendment Synopsis

Protocol Title:	A Multicenter, Double-Masked, Randomized, Dose-Ranging Trial to Evaluate the Efficacy and Safety of Conbercept Intravitreal Injection in Subjects with Neovascular Age-related Macular Degeneration
Protocol Number:	KHB-1801, Version 3.0, Amendment #1
Study Medication:	Conbercept for intravitreal (IVT) injection: 0.5 mg (10 mg/mL) and 1.0 mg (20 mg/mL)
Study Phase:	3
Primary Objective and Endpoint:	The purpose of this clinical study is to evaluate the efficacy and safety of 0.5 mg and 1.0 mg conbercept IVT injection compared with the vascular endothelial growth factor (VEGF) antagonist active control, aflibercept IVT injection (2.0 mg, Eylea®), in subjects with neovascular age-related macular degeneration (AMD). The primary objective is to determine if 0.5 mg or 1.0 mg conbercept is non-inferior to aflibercept 2.0 mg as measured by the change from baseline in best corrected visual acuity (BCVA) Early Treatment of Diabetic Retinopathy Study (ETDRS) letter score at the Week 36 visit. The primary endpoint is the mean change from baseline in ETDRS BCVA letter score at Week 36 in the study eye.
Key Secondary Objectives and Endpoints:	The secondary objectives of this study are: 1. To evaluate the difference in efficacy between conbercept doses and aflibercept, with respect to the following: • Proportion of subjects maintaining vision (i.e., losing <15 ETDRS BCVA letters) from baseline to Week 36 • Proportion of subjects gaining ≥ 15 ETDRS BCVA letters from baseline to Week 36; • Mean change from baseline in central retinal thickness (μm) measured by SD-OCT at Week 36;

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	- Proportion of subjects maintaining vision (i.e. losing <15 ETDRS BCVA letters) from baseline to Week 48; - Mean change from baseline in ETDRS BCVA letter score at Week 96. 2. To compare the safety and tolerability of conbercept doses and aflibercept, and to evaluate the pharmacokinetics and immunogenicity of conbercept doses, when feasible.
Overall Study Design: The protocol is amended by the Sponsor as of December 11, 2020. All randomized and active participants as of date of approval of this protocol at the clinical center (regulatory and ethics approval in place) will be moved from fixed dosing to each visit evaluated criteria-based capped PRN dosing regimen for all treatment arms starting at the Week 52 visit.	
Structure:	Multicenter, Multinational, Double-masked, Parallel-group, Dose-ranging, Active-controlled, Randomized trial
Duration:	A screening period of less than or equal to 14 days, followed by a treatment period of 92 weeks (last assessment at 96 weeks) with primary efficacy analysis at Week 36.
Control:	Aflibercept 2.0 mg/eye monthly IVT (at 4-week intervals) for the first 12 weeks (loading phase) followed by dosing every 8 weeks (q8w) to Week 48, to each visit evaluated criteria-based capped PRN dosing regimen starting at the Week 52 Visit through end of treatment period at Week 92.

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Dosage/Treatment Frequency:	The study will randomize approximately 1140 subjects in a ratio of 1:1:1 to receive IVT injections of 0.5 mg conbercept, 1.0 mg conbercept, or 2.0 mg aflibercept. Subjects randomized to the conbercept treatment arms will receive an IVT injection of either 0.5 mg or 1.0 mg conbercept at 4-week intervals on Day 1, Week 4, and Week 8 (loading phase). Subjects in the conbercept treatment 0.5 mg arm will begin dosing every 8 weeks (q8w) after the first three injections through Week 48. Beginning at the Week 52 visit, subjects will receive each visit evaluated criteria-based capped PRN dosing through the end of the treatment period at Week 92. Subjects in the conbercept treatment 1.0 mg arm will begin dosing every 12 weeks (q12w) after the first three injections through Week 48. Beginning at the Week 52 visit, subjects will receive each visit evaluated criteria-based capped PRN dosing through the end of the treatment period at Week 92. Aflibercept will be dosed as an IVT injection on Day 1, Week 4 and Week 8 and every 8 weeks (q8w) thereafter through Week 48. Beginning at Week 52 visit, subjects will receive each visit evaluated criteria-based capped PRN dosing through the end of the treatment period at Week 92. All subjects will receive a sham procedure at visit intervals between each scheduled IVT injection and at visits where PRN criteria for re-treatment are not met, to mask any differences in dosing frequency.
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Summary of Visit Schedule:	Subjects will be examined at Screening (≤ 14 days prior to first injection), on the day of treatment initiation, and every 4 weeks thereafter for the duration of the study. All subjects will be followed for a total of 92 weeks of study medication treatment and 96 weeks of evaluation overall.
Study Population Characteristics:	
Number of Randomized Subjects:	Approximately 1,140 subjects, 380 per arm, with a target of 340 who complete their assigned treatment for each study arm to the Week 36 visit and a target of 320 subjects in each study arm to complete the study at Week 96.
Condition/Disease:	Active neovascular AMD
Study Eye:	The study eye will be defined as an eye meeting all of the inclusion criteria and none of the exclusion criteria (i.e., the enrollment criteria). If both eyes meet the enrollment criteria, then the study eye will be defined as the eye with the lower ETDRS BCVA letter score at the time of randomization. If both eyes have the same ETDRS BCVA letter score and meet all enrollment criteria at randomization, then the Investigator will determine the study eye.
Inclusion Criteria:	1. Men and women ≥ 50 years of age at the Screening visit; 2. a. Females must be at least 1 year postmenopausal, or surgically sterilized, or, if of childbearing potential, must have a negative pregnancy test at the Screening visit; b. Women of childbearing potential must agree to use a highly effective method of contraception throughout the study (See complete list in Study Procedures Manual);

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	3. Have received no previous treatment for neovascular AMD, including laser photocoagulation and/or photodynamic therapy (PDT) and/or IVT VEGF antagonists (treatment naïve) and; 4. Have active subfoveal CNV lesions secondary to AMD (including polypoidal choroidal vasculopathy (PCV)), as evidenced by subfoveal FA leakage, or definite subfoveal fluid by SD-OCT in the study eye at Screening; 5. Have CNV that is at least 50% of total lesion size in the study eye at Screening; 6. Have a BCVA ETDRS letter score of 78 to 25 (approximately 20/32 to 20/320 equivalent) in the study eye at Screening; 7. Have ocular media (lens, cornea, vitreous) of adequate clarity to permit high quality fundus imaging; 8. Are willing and able to sign the study written informed consent form (ICF).
Exclusion Criteria:	1. Have had any prior ocular or systemic treatment (investigational or approved) or surgery for the treatment of neovascular AMD in the study eye except dietary supplements or vitamins; 2. Have participated as a subject in any interventional clinical trial within 1 month (30 days) prior to Baseline visit; 3. Have a total lesion size greater than 12 disc areas (30.5 mm²), including blood, fibrosis and neovascularization, as assessed by FA in the study eye at Screening; 4. Have a subretinal hemorrhage that is either 50% or more of the total lesion area, or blood is under the fovea and is 1 or more disc areas in size or >2.5 mm² in the study eye at Screening; 5. Have scarring or fibrosis making up >50% of total lesion in the study eye at Screening; and/or

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	scarring, fibrosis or atrophy involving the center of the fovea in the study eye at Screening; 6. Have any retinal pigment epithelial tears or rips in the study eye at Screening or upon examination at Baseline; 7. Have any vitreous hemorrhage in the study eye upon examination at Baseline or history of vitreous hemorrhage within 8 weeks prior to Screening; 8. Have any other cause of CNV, including pathologic myopia (defined per protocol as spherical equivalent of -8 diopters or more), ocular histoplasmosis syndrome, angioid streaks, inherited macular dystrophies, choroidal rupture, uveitis punctate inner choroidopathy, or multifocal choroiditis in the study eye at Screening; 9. Have a history of or clinical evidence of significant diabetic retinopathy that could impact assessment of vision or affect central vision, diabetic macular edema, or any other vascular disease other than AMD including history or clinical evidence of retinal vein occlusion affecting the study eye at Screening; 10. Have had prior pars plana vitrectomy in the study eye; 11. Have presence of a full thickness macular hole at Screening or upon examination at Baseline or a history of a full thickness macular hole in the study eye; 12. Have a history of intraocular or periocular surgery within 3 months of Baseline in the study eye, except in the case of lid surgery, which may not have taken place within 1 month of Baseline as long as it is unlikely to interfere with IVT injection; 13. Have prior trabeculectomy or other filtration surgery in the study eye; 14. Have uncontrolled glaucoma (defined as intraocular pressure (IOP) greater than or equal to
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	22 mmHg at Baseline despite treatment with more than 2 anti-glaucoma medications) in the study eye; 15. Have active intraocular inflammation in either eye at Screening or upon examination at Baseline or a history of uveitis in either eye; 16. Have active ocular or periocular infection in either eye, or a history of any ocular or periocular infection within the 2 weeks prior to Screening in either eye; 17. Have presence or history of scleromalacia in either eye; 18. Have aphakia or pseudophakia with absence of posterior capsule (unless it occurred as a result of a yttrium aluminum garnet (YAG) posterior capsulotomy) in the study eye; 19. Have had previous therapeutic radiation in the region of the study eye; 20. Have history of corneal transplant or presence of a corneal dystrophy that interferes with IOP measurements or imaging in the study eye; 21. Significant media opacities, including cataract, in the study eye that, in the opinion of the Investigator, could require either medical or surgical intervention during the study period; 22. Have any concurrent ocular condition in the study eye that, in the opinion of the Investigator, could either increase the risk to the subject beyond what is to be expected from standard procedures of intraocular injection, or that otherwise may interfere with the injection procedure or with evaluation of efficacy or safety during the study; 23. Have any evidence by medical history, physical examination or clinical laboratory testing at Screening or Baseline that shows reasonable suspicion of a disease or condition that contraindicates the use of study medication (conbercept or aflibercept) or that might affect
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	interpretation of the results of the study or render the subject at high risk for treatment complications; 24. Have any use of long acting intraocular steroids, including implants, within 6 months prior to Day 1, Baseline; 25. Have any known allergy to povidone iodine or known serious allergy to the fluorescein sodium for injection in angiography; 26. Any history of known contraindications indicated in the Food and Drug Administration (FDA)-approved label for the active control; 27. If female, be pregnant (positive urine pregnancy test at Screening) or breastfeeding.
Study Formulations:	Two concentrations of conbercept will be studied: 0.5 mg dose (10 mg/mL) and 1.0 mg dose (20 mg/mL) for IVT injection Aflibercept will be the active comparator for IVT injection (Eylea®: 40 mg/mL, 2.0 mg/eye dose).
Evaluation Criteria	
Efficacy Measures:	• BCVA as measured by ETDRS procedure with manifest refraction • Retinal thickness and anatomy and lesion characteristics as measured by SD-OCT and imaging modalities

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Safety Measures:	• Adverse event reporting • Slit-lamp biomicroscopy • IOP • Dilated indirect ophthalmoscopy • SD-OCT • Laboratory analyses of blood and urine • Immunogenicity to conbercept in serum (sub-group) • Specular microscopy (sub-group) • Urine pregnancy testing at Screening
Other:	• Patient-reported outcome: NEI-VFQ 25 • Pharmacokinetics of conbercept (sub-group), where feasible

Sample Size

The study will randomize approximately 1140 subjects in a 1:1:1 ratio to the three study treatment arms.

The sample size is driven by the expectation that at least 300 subjects in each treatment group will complete 96 weeks on their assigned treatment.

The primary efficacy endpoint is the mean change in ETDRS BCVA letter score from baseline at Week 36. If all three treatments are equally effective and if 90% percent of subjects in each treatment group complete the first 36 weeks of treatment, roughly 340 subjects in each group are expected to finish the study on their assigned study medication. Under these conditions, if this “completers” population data were to be used to test the primary outcome, the study would have 92% power to conclude non-inferiority to aflibercept of either the 0.5 mg conbercept dose or the 1.0 mg conbercept dose in the mean change from baseline in BCVA letters at Week 36, assuming a 2-sided Type I error rate of 0.025 (using Bonferroni correction for 2 comparisons with aflibercept group) for each test and a common standard deviation (SD) of 14.5 letters. If the two tests were independent, the probability that at least one conbercept dose will be shown non-inferior to aflibercept would be roughly 99%. Because the two tests are not independent (they share the same control group), the power to show at least one conbercept dose non-inferior to aflibercept will be between 92% and 99%.

The actual statistical test will use a longitudinal model which is expected to decrease the width of the confidence interval.

Endpoint Analysis

Primary and secondary efficacy endpoints in the study eye will be summarized using appropriate summary statistics, including appropriately sized confidence intervals (CIs) for each treatment and will be compared between each of the 2 test arms and the control arm. This Statistical Analysis Plan (SAP) describes the methods to be used.

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Abbreviations

AE	adverse event
AMD	age-related macular degeneration
ANOVA	analysis of variance
AR(1)	autoregressive model of order 1
BCVA	best correct visual acuity
CI	confidence interval
CNV	choroidal neovascularization
CRO	Contract Research Organization
CSR	clinical study report
CTCAE	Common Terminology Criteria for Adverse Events
eCRF	electronic Case Report Form
ETDRS	Early Treatment Diabetic Retinopathy Study
EU	Europe
FA	fluorescein angiography
FAS	full analysis set
FDA	Food and Drug Administration
IB	Investigator's Brochure
ICH	International Conference on Harmonization
IDMC	Independent Data Monitoring Committee
IOP	intraocular pressure
IVT	intravitreal
IxRS	Interactive Response System
MedDRA [®]	Medical Dictionary for Regulatory Activities
mg	milligram
ml	milliliter
mmHg	millimeters of mercury
NEI-VFQ-25	National Eye Institute Visual Function Questionnaire
OCT	optical coherence tomography
PED	pigment epithelium detachment
PP	per protocol

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PRN	pro re nata
PT	preferred term
q8w	Every 8 weeks
q12w	Every 12 weeks
RPE	Retinal pigment epithelium
SAE	Serious Adverse Event
SAP	Statistical Analysis Plan
SD	Standard deviation
SD-OCT	Spectral domain optical coherence tomography
SOC	System organ class
TEAE	Treatment emergent Adverse Event
US	United States

1. Introduction

This statistical analysis plan (SAP) defines the methods and analyses that Chengdu Kanghong Biotechnology Company, henceforth referred to as Kanghong, will use to analyze data from the KHB-1801 clinical research study. This SAP presents details of the planned statistical methods addressing the study aims.

The amended SAP is based on Amendment 1 of the KHB-1801 study protocol dated December 11, 2020. If the protocol is subsequently amended, this SAP may be amended as well. The SAP describes the methods to be used in detail. If the SAP and the protocol disagree, the details and methods of the SAP will prevail.

This SAP complies with the International Conference on Harmonization (ICH) guidance and relevant Food and Drug Administration (FDA) guidance.

2. Investigational Plan

2.1. Study Design

Protocol KHB-1801 is a multi-center, double-masked, parallel group, dose-ranging, active controlled, randomized evaluation of intravitreal (IVT) conbercept in neovascular age-related macular degeneration (AMD).

Subjects with neovascular AMD who provide informed consent and meet enrollment criteria (defined as meeting all inclusion criteria and none of the exclusion criteria) will be randomized in a 1:1:1 ratio to one of the following three treatment arms.

- Conbercept 0.5 mg (10 mg/mL) for IVT injection
- Conbercept 1.0 mg (20 mg/mL) for IVT injection
- Afibercept 2.0 mg (40 mg/mL) for IVT injection

Details of the three treatment arms follow:

- 0.5 mg conbercept at Day 1, Week 4, and Week 8 (three injection loading dose followed by dosing every 8 weeks (q8w) after the first three injections through Week 48. Beginning at the Week 52 visit, subjects will receive each

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visit evaluated criteria-based capped PRN dosing through the end of the treatment period at Week 92;

- 1.0 mg conbercept at Day 1, Week 4, and Week 8 (three injection loading dose), and treated every 12 weeks thereafter (1.0 mg, q12w) through Week 48. Beginning at Week 52 visit, subjects will receive each visit evaluated criteria-based capped PRN dosing through the end of the treatment period at Week 92;
- Aflibercept (2.0 mg/eye, Eylea®) at Day 1, Week 4, and Week 8 (three injection loading dose), and treated every 8 weeks thereafter (2.0 mg, q8w) through Week 48. Beginning at Week 52 visit, subjects will receive each visit evaluated criteria-based capped PRN dosing through the end of the treatment period at Week 92.

As of December 11, 2020, the Sponsor has amended the protocol regarding the dosing schedule after the Week 48 Visit. Beginning at Week 52, all randomized participants will receive evaluation at each visit for criteria-based capped PRN dosing of the assigned treatment arm through the Week 92 Visit.

All enrolled subjects will receive treatment according to the protocol's schedule of visits and procedures (see Appendix 1). A total of approximately 1140 subjects (380 in each study arm) will be randomized at sites globally.

For each subject, the study eye (one study eye per subject) is defined as the eye meeting the enrollment criteria. If both eyes meet the enrollment criteria, the study eye will be defined as the eye with the lower best corrected visual acuity (BCVA) letter score as assessed by the Early Treatment Diabetic Retinopathy Study (ETDRS) chart and protocol at the time of randomization. Hereafter, the SAP will use BCVA to refer to "BCVA as assessed by ETDRS". If both eyes have the same BCVA letter score at the time of randomization, the Investigator will determine the study eye.

2.2. Study objectives and outcome measures

The KHB-1801 study is designed to evaluate the efficacy and safety of 0.5 mg and 1.0 mg conbercept IVT injection compared to 2.0 mg aflibercept in subjects with active neovascular AMD.

See Table 1 for the study objectives and outcome measures.

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Table 1. Study objectives and outcome measures

<i>Objective and outcome measure in study eye</i>
Primary
Objective: To determine if 0.5 mg or 1.0 mg conbercept is non-inferior (non-inferiority margin of -4 letters) to aflibercept 2.0 mg with respect to the change from baseline in ETDRS BCVA letter score at Week 36. Outcome measure: Mean change in ETDRS BCVA letter score from baseline at Week 36
Secondary
Objective 1: To evaluate the difference in efficacy between the conbercept doses and aflibercept with respect to the following outcome measures in the study eye: • Proportion of subjects maintaining vision (i.e., losing <15 letters in ETDRS BCVA) from baseline to Week 36 • Proportion of subjects gaining at least 15 letters from baseline to Week 36 • Mean change in retinal thickness as measured by spectral domain optical coherence tomography (SD-OCT) from baseline at Week 36 • Proportion of subjects maintaining vision (i.e., losing < 15 letters in ETDRS BCVA) from baseline to Week 48 • Mean change in ETDRS BCVA letter score from baseline to Week 96 Objective 2: To compare the safety and tolerability of conbercept doses and aflibercept with respect to the follow outcome measures using: • Slit-lamp biomicroscopy • IOP and Dilated indirect ophthalmoscopy • SD-OCT • Laboratory analyses of blood and urine • Adverse event reporting • Specular microscopy (in a subgroup of subjects)
Exploratory
To evaluate differences between the conbercept doses and aflibercept with respect to the following outcome measures in the study and fellow eye: • Mean change from baseline in ETDRS BCVA letter score at all study visits, except for Week 96 • Proportion of subjects maintaining vision (i.e., losing <15 ETDRS BCVA letters) at each visit, except for Week 36 and Week 48 • Proportion of subjects with 20/40 vision equivalent (74 to 78 BCVA letters) or better at Week 36, 48, 96, and each study visit • Proportion of subjects losing 5, 10, and 15 letters or more at every visit from Week 36 to Week 96 • Proportion of subjects gaining 5, 10, and 15 letters or more at every visit from Week 36 to Week 96 (except for proportion of subjects gaining 15 letters or more at Week 36) • Mean and/or mean change from baseline in all available continuous anatomical parameters obtained from fluorescein angiography (FA), for example, the choroidal neovascularization (CNV) area, at Week 36 and Week 96 • Categorical anatomical parameters obtained from FA, for example, the proportion of patients with leakage at Week 36 and Week 96 • Mean change from baseline in anatomic measures, as measured by OCT, including, for example, total macular volume, persistent fluid, pigment epithelium detachment (PED) height (or volume) as assessed by SD-OCT at Week 36, 48, 96, and each study visit and central retinal thickness at Week 48 and 96 and each study visit • Categorical anatomical parameters based on OCT, for example, the proportion of patients with dry retina at each Week 36, 48, 96, and each study visit • Mean change from baseline in visual function as assessed by the National Eye Institute Visual Function Questionnaire (NEI-VFQ)-25 at Week 36, 48, and 96

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- Analysis and distribution of re-treatment in the PRN phase
- Analysis and distribution of re-treatment criteria status from Week 52 to Week 92

Notes: BCVA is BCVA as assessed by ETDRS methodology; “maintaining vision” is defined as losing < 15 ETDRS BCVA letters

2.3. Randomization and masking

Randomization will occur at Visit 2.

The Investigator or designee will contact the Interactive Response (IxR) System to randomize subjects who meet all enrollment criteria. Allocation of study treatment will be completed through the IxR System that determines which kit to assign a subject. At each subsequent visit, the Investigator designee will access the IxR System for the subject's kit assignment for that visit.

Because of the requirement for masking as well as sham procedures, a delegated unmasked Investigator at each clinical center, not involved in or delegated to any other study assessments, will perform the study medication injection or sham procedure as well as the immediate pre- and post-dose safety assessments. Unmasked personnel will also be responsible for the receipt, storage, monitoring and disposition of all study medication and related materials.

Investigators will use supply of the sham procedure kits to maintain masking of study subjects and staff on those visits that do not require a per protocol IVT injection of study medication. The IxR System will assign the subject the appropriate kit at each visit and study staff must dispense the correct kit.

The Study Procedures Manual will contain details concerning masking through key protocol endpoints.

2.4. Safety Monitoring

An Independent Data Monitoring Committee (IDMC) will periodically review safety data from the study to ensure subject safety. The IDMC will focus on subject safety, and will be reviewing masked safety data. The charter of the IDMC will specify that the IDMC may ask for specific subject or subjects unmasking as part of individual subject case safety review. No IDMC data review will be shared with the sponsor or anyone involved in the masked operations of the study. As stated in the IDMC charter, no futility or efficacy analysis is planned as part of the IDMC review; however, the IDMC will meet multiple times during the study. An alpha of 0.0001 will be allocated to each IDMC

assessment. If there are 10 meetings of the IDMC with looks at the visual acuity data, there will be an alpha of 0.049 ($=0.05-10\times0.0001$) for the final analysis.

The charter of the IDMC will describe in detail the review of safety data.

3. Study Schedule and Timepoints

Subjects are expected to attend 26 visits over the period of approximately 96 weeks of participation in the study. They are examined at screening (≤ 14 days prior to first injection), on the day of treatment initiation, and every 4 weeks thereafter. All subjects are scheduled to be followed for a total of 92 weeks of drug treatment and 96 weeks of evaluation. See Appendix 1, for the schedule of events.

3.1. Study days

For the purpose of the analysis, study days will be calculated relative to the date of randomization, which will be designated as Day 1 (Visit 2). Positive study days will be measured forward in time from randomization while negative study days will be measured backward from randomization (e.g., Day 2 will represent the day after randomization while Day -1 will represent the day before randomization).

3.2. Unscheduled visits

Unscheduled visits may be performed to ensure safety of the subject. The study Investigator or designee should record all information gathered at unscheduled visits on the Unscheduled Visit/Early Exit Visit pages of the Source Document and corresponding electronic Case Report Form (eCRF). If a randomized subject does not attend a scheduled visit, the eCRF pages for the missed visit will be skipped. All reasonable efforts should be made to schedule subjects for an Exit Visit to complete Exit Procedures.

3.3. Rescue Treatment

No rescue treatment is allowed. A subject who is in need of rescue in the opinion of the Principal Investigator may be exited from the study and early termination procedures will be completed.

4. Analysis Populations

4.1. Full analysis set population

The full analysis set (FAS) population will be comprised of all randomized subjects. Analyses using the FAS will classify subjects according to the treatment to which they were randomized. The FAS analyses will be considered the primary analysis population for the primary and secondary outcomes.

4.2. Per-protocol analysis population

The per protocol (PP) population will include all FAS subjects who received at least five doses of study medication and attended at least seven scheduled visits during the first 36 weeks of the study, had not missed two consecutive injections before 36 weeks, and who had no major protocol violation likely to seriously affect the primary outcome of the study as judged by a masked evaluation prior to the unmasking of the study treatment.

Analyses using the PP population will group subjects according to treatment actually received. The PP population will exclude any subject who, in error, received different treatments on different visits. The PP analyses will be considered sensitivity analyses.

4.3 Modified per-protocol analysis population

The modified per protocol (mPP) population will include all subjects in the FAS who received all three loading doses (Day 1, Week 4, and Week 8) and the Week 32 injection, and who had no major protocol violation likely to seriously affect the primary outcome of the study as judged by a masked evaluation prior to the unmasking of the study treatment. The mPP analyses will be considered sensitivity analyses.

4.4. Safety population

The safety population will include all randomized subjects who received any study medication. In the safety analysis, no data exclusion is allowed because of protocol violations. Safety analyses will group subjects according to the treatment actually received. Subjects who receive, in error, different treatments on different visits will be assigned the

group corresponding to the treatment most frequently received.

5. Sample Size Determination and Power Calculation

The primary hypothesis underlying the study is that each dose of conbercept will be non-inferior to aflibercept when the non-inferiority margin of the BCVA letter score measured by ETDRS is defined as -4 letters.

The sample size of the study is driven by the expectation that at least 300 subjects in each conbercept group will complete 96 weeks of evaluation on their assigned treatment.

Experience from the VIEW 1 and VIEW 2 studies suggest that 16% of the subjects in each study group are likely to fail to finish 96 weeks of treatment on their assigned study drug [1]. If the study randomizes 380 participants in each treatment group and 16% participants in a treatment group fails to complete the planned 96 weeks on their assigned drug, that treatment group will have approximately 320 participants completing. Even if 20% of participants failed to complete 96 weeks of treatment in any or all of the treatment groups, the study group will have slightly over 300 completed subjects.

Therefore, the study will randomize approximately 1140 subjects in a 1:1:1 ratio to the three treatment groups. If all three treatments are equally effective and if 90% percent of subjects in each treatment group complete the first 36 weeks of treatment, roughly 340 subjects in each group are expected to finish the study on their assigned study medication. Under these conditions, if this “completers” population data were to be used to test the primary outcome, the study would have 92% power to conclude non-inferiority to aflibercept of either the 0.5 mg conbercept dose or the 1.0 mg conbercept dose in the mean change from baseline BCVA letters at Week 36, assuming a 2-sided Type I error rate of 0.025 (using Bonferroni correction for 2 comparisons with aflibercept group) for each test and a common standard deviation (SD) of 14.5 letters.

If the two tests were independent, the probability that at least one conbercept dose will be shown non-inferior to aflibercept would be roughly 99%. Because the two tests are not independent (they share the same control group), the power to show at least one conbercept dose non-inferior to aflibercept will be between 92% and 99%.

These calculations assume use of a t-test to compare each conbercept group to aflibercept

with respect to mean change in BCVA letter score at Week 36. As described below, the actual test will use a longitudinal model, which is expected to increase the power. The model will allow precise estimates of the variability of BCVA; however, the statistical tests will be based on the estimated mean change from baseline to Week 36. Moreover, the FAS population, not a per protocol population, will be used with trimmed mean to handle subjects who stop the study treatment early. The power calculations above are approximately valid for the actual planned analyses.

In summary, the primary non-inferiority endpoint will be tested against a two-sided, adjusted Type I error rate of 0.025 with a non-inferiority margin of -4 letters. That is, a dose regimen of conbercept will be considered non-inferior to aflibercept if the lower bound of the 97.5% confidence interval of the mean difference of the number of letters read between two drug groups (conbercept – aflibercept) is above -4. Use of the 97.5% confidence interval for the two doses yields a 95% coverage probability because there will be two confidence intervals defined by the two doses. This coverage for this interval will be further adjusted based on the number of IDMC meetings.

6. Violations and Deviations

Important protocol deviations will be summarized by randomized treatment group. Sensitivity analyses for the primary and secondary efficacy outcomes will be performed if the deviations have the potential to have a non-trivial impact on the analysis.

The following general categories are examples of important violations and deviations:

- Randomized subjects who did not receive any IVT injection;
- Subjects who deviated from any entry criterion;
- Visits outside of protocol visit windows;
- Incomplete assessments;
- Use of prohibited concomitant medications;
- Randomized subjects who received their study treatment at an incorrect dose or received a study treatment different from the one to which they were randomized.

Prior to each database lock, members of the study team will review the data for each

subject to assess whether a violation or deviation has occurred in a masked fashion. The Data Management Plan will describe the process planned for those reviews.

7. Statistical Analysis: General Considerations and Conventions

Descriptive and inferential statistics will be used to summarize results of protocol KHB-1801. Standard descriptive statistics, such as mean, standard deviation, quartiles and other relevant percentiles, minimum, and maximum, will be calculated for continuous variables. For discrete variables, descriptive analyses will be based on numbers of subjects and related percentages.

For binomially distributed variables, the treatment effect will be measured as differences in proportions. For continuous random variables, analysis of variance (ANOVA) or longitudinal models will be used for testing treatment differences or effects.

Statistical testing will be performed in a manner that maintains the family-wise Type I error rate at 0.05 and the coverage probability of confidence intervals at 95%. The sections of this SAP that describe each outcome provide details on the Type I error rate.

All data listings, summaries, and statistical analyses will be generated using SAS® Version 9.4 or higher or other validated software.

The following sections describe the analyses to be used for the study.

7.1. Definition of time

For visits (or events) that occur on or after randomization, time is calculated in days as:

$$\text{time} = \text{visit (or event) date} - \text{date of randomization} + 1$$

For visits (or events) that occur prior to randomization, time is calculated in days as:

$$\text{time} = \text{visit (or event) date} - \text{date of randomization}$$

For listings (e.g., for adverse events), the number of days since first (or last) dose is defined as:

$$\text{days since first (or last) dose} = \text{event date} - \text{date of first (or last) dose} + 1$$

For summaries that present distribution of time expressed in weeks and months, weeks will be defined as days divided by seven and months as days divided by 30.4.

7.2. Missing and partial data

For the analysis of safety variables, only partial dates may be imputed; otherwise, missing data will be treated simply as missing. The algorithms for imputation of partial dates depend upon the parameter (see Appendix 2).

8. Subject Characteristics, Disposition, and Exposure to Study Drug

8.1. Demographic and baseline characteristics

Descriptive statistics will be used to describe demographic and baseline characteristics of the subjects. Sample size, mean, standard deviation, quartiles, minimum, and maximum will be used to summarize continuous variables, while number and percentages of subjects will be used for categorical variables. The following demographic and baseline characteristics will be summarized: age category, gender, ethnicity, race, country of location of study site, and iris color for the study eye.

Age in years will be calculated as the integer portion of the following:

$$[(\text{date of randomization} - \text{date of birth}) + 1] / 365.25.$$

Unless otherwise stated, percentages will be calculated relative to the number of subjects in the FAS. All demographic data will be listed.

Summary tables will describe baseline ocular characteristics for each treatment group (e.g., CNV lesion size, OCT fluid, fluid location, type of CNV).

Medical history will be coded using the Medical Dictionary for Regulatory Activities (MedDRA®) and summarized, for each treatment group, by system organ class (SOC) and preferred term (PT). Medical history may also be listed for all subjects in the FAS.

Medical history may be sorted by treatment group and descending overall frequency, by SOC and PT, in the summary tables. Medical history data may be sorted by treatment

group, subject number, start date, SOC and PT. Medical history will be presented separately for ocular and non-ocular events.

8.2. Subject disposition

The study has three times of evaluation of primary or secondary outcomes: Week 36, Week 48, and Week 96. At each of these times, the disposition of all subjects will be listed and summarized by treatment group. Tables will present the number and percentage of subjects who were screened, randomized, received treatment, or withdrew from the study early by treatment arm and overall. For subjects who were randomized but withdrew from the study early, the number and percentage withdrawing by reason will be presented by treatment arm and overall.

The Case Report Form (CRF) lists the following reasons why subjects may discontinue treatment before completion of the study:

- Adverse event (AE) or serious adverse events (SAE)
- Protocol violation
- Administrative reason (e.g., inability to continue, lost to follow up)
- Sponsor termination of study
- Voluntary withdrawal or withdrawal of consent

In addition, any subject may be discontinued for any sound medical reason at the discretion of the Investigator.

8.3. Treatment exposure

As described in Section 2.1, all participants are to receive three loading doses (Day 1, Week 4, and Week 8) of their assigned study medication.

As of December 11, 2020, the Sponsor has amended the protocol regarding the dosing schedule after the Week 48 Visit. Beginning at Week 52, all randomized participants will receive an evaluation at each visit for criteria-based capped PRN dosing of the assigned treatment arm through the Week 92 Visit.

Participants in the conbercept 0.5 mg group and the aflibercept 2.0 mg group are scheduled for treatment every 8 weeks (q8w) after the loading dose through Week 48. Beginning at the Week 52 visit, subjects will receive an evaluated criteria-based capped

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PRN dosing at each visit through the end of the treatment period at Week 92.

Participants in the conbercept 1.0 mg group are scheduled for treatment every 12 weeks (q12w) after the loading dose through Week 48. Beginning at the Week 52 visit, subjects will receive an evaluated criteria-based capped PRN dosing at each visit through the end of the treatment period at Week 92.

Administration of conbercept and aflibercept will be summarized descriptively by randomized treatment arm for each protocol-specified injection. The presentations will also show how many subjects have stopped study treatment.

For each treatment period (through 36 weeks, through 48 weeks, through 96 weeks, and throughout the PRN period which may cover Week 52 through Week 96), the distribution of number of injections will be summarized by treatment arm.

8.4. Concomitant medications and therapies

Information on concomitant medication may be summarized by generic name as reported on the concomitant medication CRF. Prior and concomitant medications will be coded by the Anatomical Therapeutic Chemical (ATC) classification system according to the World Health Organization Drug Dictionary (WHO-DD).

The number and percentage of subjects who took at least one drug within each generic type will be presented. Subjects will only be counted once if they are taking the same generic medication more than once.

9. Baseline, Post-baseline, and Unscheduled Measurements

Baseline is defined as the last non-missing measure prior to randomization (Day 1, Visit 2), unless otherwise specified in Table 2 below. Unless otherwise specified, if a subject has more than one baseline value, the value recorded closest to the date and time of randomization prior to the first injection will be used as the baseline measurement.

If a parameter is not collected on Day 1, baseline will be defined as the screening measure (Visit 1). The clinical study report (CSR) will include listings of all repeated measurements collected prior to first dose indicating whether they were included in the calculation of

baseline.

If a laboratory result is retested and the laboratory subsequently considers the original result valid, the original result will be used and the second (i.e., ‘retested’) result will be ignored. Otherwise, if the laboratory detects an error through the retesting, the retested result will be used as the scheduled measurement and the original result will be ignored.

For post-baseline measures, specific time points will be calculated relative to randomization, rather than solely on the visit labels in the clinical database.

Results of unscheduled tests will be presented in data listings, but they will not be included in summaries of safety for the analysis of change in BCVA letter score unless otherwise specified.

Specifications of baseline and follow-up measurements for each procedure are described in Table 2. For further details, see Appendix 1.

The set of table shells that will be prepared prior to database lock will describe in detail how these various measurements will be presented.

Table 2. Baseline measurements

Procedure/measurement	Baseline
Routine blood & urinalysis	Visit 1
BCVA (ETDRS), both eyes	Visit 2
Blood sampling for PK and immunogenicity testing ^a	Visit 1
Slit lamp biomicroscopy, both eyes	Visit 2
IOP assessment, both eyes	Visit 2
SD-OCT	Visit 2
Indirect dilated ophthalmoscopy, both eyes	Visit 2
Fundus photo & FA, both eyes	Visit 2
Specular microscopy (sub-group only)	Visit 1
Visual function questionnaire (NEI-VFQ-25)	Visit 2

a. subgroup only

Note: Visit 1 is Screening; Visit 2 is Day 1.

9.1. Blood and urine sample collection for laboratory analyses

The schedule of events (Appendix 1) displays the timing of laboratory tests. Laboratory data (absolute values and absolute change from baseline) may be summarized by visit, treatment group and overall. All summaries will be based on results in International System of Units (SI units); conversion will be performed prior to the transfer to the biostatistics team.

All laboratory values may be categorized according to their normal ranges, where normal ranges exist. Shift tables versus baseline may be created.

Toxicity grading following the NCI CTCAE grades version 4.03 will be derived for laboratory variables where applicable. If this results in the criteria for more than one grade being met, the highest (worst) CTCAE grade may be assigned.

Summary statistics (mean, median, standard deviation, appropriate percentiles, minimum, maximum and number of observations) will be presented for all continuous assessments. In general, any quantitative assessments will be summarized for all subjects using the number and percentage of subjects with the given result. Percentages will be calculated relative to the number of subjects with non-missing data.

Shift tables from baseline CTCAE toxicity to the maximum grade on treatment for each parameter may be provided. Subjects with both a non-missing baseline and at least one non-missing post-baseline grade will be included in the shift tables. Unscheduled data

will be included in “worst post-baseline” summaries which will capture a worst case across all scheduled and unscheduled visits after the first dose of study treatment.

9.2. Ocular assessments

Ocular assessments will be listed, summarized in tables, and presented in figures as appropriate.

9.2.1. BCVA

Visual function of the study and fellow eye will be assessed using the best corrected ETDRS protocol starting at 4 meters. Visual Acuity examiners must be certified to ensure consistent measurement of BCVA. Detailed instructions for obtaining BCVA will be provided in the study-specific BCVA manual.

9.2.2. *Slit lamp biomicroscopy*

External examination and biomicroscopy will be performed using a slit lamp. Abnormal findings that are clinically significant will be described. The eyelids, cornea, conjunctiva, anterior chamber, iris, and lens will be examined at each visit. Slit lamp biomicroscopy results will be summarized for each treatment group for the study eye and fellow eye by visit using discrete summary statistics. Shifts from baseline including normal to abnormal, not clinically significant; abnormal, not clinically significant to abnormal clinically significant; and normal to abnormal, clinically significant will also be presented using counts and percentages.

9.2.3. *Intraocular pressure*

Intraocular pressure (IOP) may be measured after the biomicroscopic exam is complete and will be measured prior to pupil dilation. See the protocol for timing of IOP measurements. Each IOP measurement is to be recorded.

For each visit, IOP will be summarized by treatment group separately for the study and the fellow eye. The numbers and percentages of eyes with $IOP \geq 30$ mmHg and the numbers and percentages of eyes with an increase in $IOP \geq 10$ mmHg from baseline will also be summarized.

9.2.4. *Optical coherence tomography*

Spectral domain optical coherence tomography (SD-OCT) will be utilized to assess

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retinal characteristics and will be conducted at every visit. OCT imaging data will be transmitted to a central reading center for independent analysis.

Mean change in anatomic measures including retinal thickness, total macular volume, and persistent fluid will be assessed by optical coherence tomography (OCT).

Abnormal findings that are clinically significant will be described. Results will be summarized for each treatment group for the study eye and fellow eye by visit using discrete summary statistics.

9.2.5. *Indirect ophthalmoscopy*

Dilated indirect ophthalmoscopy will be performed at every visit to assess the vitreous, macula, optic nerve and peripheral retina of both eyes. Findings will be graded as normal; abnormal, not clinically significant; or abnormal, clinically significant. Results will be summarized for each treatment group for the study eye and fellow eye by visit using discrete summary statistics. Shifts from baseline including normal to abnormal, not clinically significant; abnormal, not clinically significant to abnormal clinically significant; and normal to abnormal, clinically significant will also be presented using counts and percentages.

9.2.6. *Fundus photography and fluorescein angiography*

Color fundus photographs and fluorescein angiography of both eyes to evaluate retinal anatomy will be taken at Screening, Week 36, and Week 96. Fundus photography will also be taken in the study eye only at Week 24, Week 48, and Week 72. Abnormal findings that are clinically significant will be described. Results will be summarized for each treatment group for the study eye and fellow eye by visit using discrete summary statistics.

9.3. *Specular microscopy*

Only sites selected for the specular microscopy sub-study will perform specular microscopy at Screening, Week 36, and Week 96. Results will be summarized as described in Section 9.2.2.

9.4. Pharmacokinetics

Descriptive statistics will be used to analyze the pharmacokinetics data.

9.5. Immunogenicity

Descriptive statistics will be used to analyze the immunogenicity data.

10. Handling Multiplicity of the Efficacy Analyses

This study, which investigates two doses of conbercept, has two primary hypotheses (one for each dose) and 10 secondary hypotheses (five for each dose).

The study has three time points of measurement of measurement associated with the primary and secondary hypotheses (36 weeks for the primary hypotheses and three of the secondary hypotheses for each dose; 48 weeks and 96 weeks for one secondary hypothesis for each dose). A Bonferroni correction will be used to handle multiplicity of the two primary non-inferiority hypotheses.

Each secondary outcome will be tested for non-inferiority of a conbercept dose against aflibercept using a Bonferroni correction that accounts for the five secondary outcomes and the two doses. Only if a dose shows non-inferiority for the primary outcome will the secondary outcomes for that dose be tested.

11. Primary Efficacy Analysis

As described in Section 2.2, the primary efficacy outcome is the mean change in BCVA letter score from Baseline to Week 36. The study has two non-inferiority primary hypotheses, one for each dose. Specifically, each conbercept dose will be evaluated against aflibercept with a non-inferiority margin for the mean of -4 letters.

The statistical literature on selecting the proper population for testing non-inferiority has been controversial. Some authors recommend using a per protocol population on the grounds that non-compliance to study drug biases the results toward no difference between groups, i.e., toward non-inferiority. Others argue that because per protocol analyses do not respect randomization, they are inherently problematic and therefore non-inferiority studies should use the intent-to-treat population; these authors then state that a study with more than

a small amount of non-compliance or drop out is difficult to interpret.

The current standard approach to analysis has been to perform both a per protocol and an intent-to-treat analysis; the study will have shown non-inferiority only if both analyses show non-inferiority [2-5].

Given the divergent views concerning the appropriate population for non-inferiority, the approach to analysis of this study will adopt the stance of the National Research Council's monograph on missing data [6]. The intent-to-treat population will be used for the non-inferiority tests of the primary outcomes with careful consideration of how to handle missing data.

11.1. Primary efficacy outcome

The primary efficacy outcome, mean change from baseline in BCVA at Week 36 in the study eye, will be summarized using continuous summary statistics for each treatment group.

The primary efficacy analysis for each conbercept dose will test the difference in the mean change from baseline in BCVA letter score in study eyes at Week 36; the 97.5% CIs around the differences between the mean change for each conbercept arm and the aflibercept arm will be calculated from a longitudinal model with change from baseline in BCVA letter score at each post-treatment visit through Week 36 as the response variable, baseline BCVA letter score as a covariate, visit (as a categorical variable), and treatment×visit interaction. A trimmed mean will be used to handle subjects who do not complete 36 weeks of treatment. The choice of 97.5% CIs reflects the use of the Bonferroni correction for multiplicity (see Section 5).

The primary pair of statistical tests will compare each dose arm of conbercept (0.5mg or 1.0 mg) to aflibercept with respect to the mean change from baseline to Week 36 in BCVA score.

Within-subject correlations in BCVA score over time will be modeled using an unstructured covariance structure applying the default method in SAS for computing the denominator degrees of freedom for the tests of fixed effects will be used. Because an unstructured covariance structure will be used with a REPEATED statement, all effects will be assigned—

the between-subject degrees of freedom to provide better approximations to the relevant sampling distributions.

In the unlikely situation that this model does not converge, an analysis of covariance model will be used.

For each dose of conbercept, the non-inferiority null hypothesis specifies that at Week 36 the difference in mean change between conbercept and aflibercept is greater than -4 letters against the alternative that the difference is less than or equal to -4 letters.

Each of these two hypotheses, one for each dose, will be tested at a 1-sided Type I error rate of 0.0125. Two-sided 97.5% CIs will be used for each testing to maintain an overall two-sided alpha level of 0.05 for the two hypotheses being tested.

The study will be considered a success if either dose of conbercept shows statistically significant evidence of non-inferiority to aflibercept of -4 letters.

Subjects who do not complete 36 weeks of treatment will be assumed to have experienced bad outcomes. The analysis of Week 36 BCVA will be based on the trimmed mean method introduced by Permutt and Li (2017).

Appendix 4 provides details of the planned methodology, including the pseudo SAS code.

11.2. Secondary outcome analyses

Each of the five secondary outcomes will be tested with a two-sided critical p-value equal to 0.005 with an additional adjustment for the number of IDMC meetings as described in Section 2.4. This value comes from a Bonferroni p-value for each pair (i.e., the two doses) divided by five to represent a Bonferroni correction for the five pairs. The secondary analyses will use the Full Analysis Set. Appendix 4 presents the pseudo SAS code for these analyses along with the specified non- inferiority margins for each secondary outcome. The details of the adjustment to the significance level to account for the IDMC meetings are provided in an addendum to this SAP. Note that nominal significance levels for secondary outcomes are used if the primary outcome is not significant.

12. Safety Analyses

The safety population will be used for all safety analyses. All recorded safety parameters will be listed. See Appendix 1 for the schedule of times when safety assessments are to be performed. The following sections describe the methods to be used to summarize data on safety.

12.1. Ocular and Non-ocular adverse events (AEs)

An AE is defined as any untoward medical occurrence associated with the administration of the study medication in a subject, whether or not the event is considered drug-related. An AE can therefore be any unfavorable and unintended sign (e.g., an abnormal laboratory finding), symptom, or disease occurring after the subject started dosing with the study drug, regardless of the causality. Any pre-existing medical condition that worsens after administration of the study medication will also be considered a new AE.

Adverse events (both elicited and observed) will be monitored throughout the study and documented on the appropriate CRF. AEs will be classified as ocular or non-ocular and will be coded by MedDRA and summarized by system organ class (SOC), and preferred term (PT). Adverse events will be summarized by subject, not event. The Investigator will promptly review each AE for accuracy and completeness, and classify each AE according to its intensity, its relationship to study drug or injection procedure, and its seriousness.

All AE presentations will summarize treatment-emergent adverse events (TEAEs) defined as adverse events with onset on or after randomization or onset prior to randomization but with worsened severity post-randomization. AEs without a severity rating that are reported post-randomization will be considered treatment-emergent.

The following presentations of treatment-emergent adverse events will be generated:

- Serious adverse events;
- All adverse events;
- Adverse events suspected to be related to the study medication
- Adverse events suspected to be related to the injection procedure
- Adverse events leading to study discontinuation.

Separate summaries of treatment-emergent adverse events will present number of events

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and number of events per study month. The number of study months for each subject will be computed as the time from randomization to death or 30 days after study termination or study withdrawal.

12.1.1. Severity

Severity of an AE is defined as a qualitative assessment of the degree of intensity of an AE as determined by the Investigator or reported to him/her by the subject. The assessment of severity is made irrespective of relationship to study treatment or seriousness of the event and should be evaluated according to the following scale:

- *Mild*: Event is noticeable to the subject but is easily tolerated and does not interfere with the subject's daily activities.
- *Moderate*: Event is bothersome, possibly requiring additional therapy, and may interfere with the subject's daily activities.
- *Severe*: Event is intolerable, necessitates additional therapy or alteration of therapy, and interferes with the subject's daily activities.

12.1.2. Relationship to study medication and injection procedure

The Investigator should use the following two categories to classify the relationship of each AE to the study medication and/or to the injection procedure:

- *Suspected*: A reasonable possibility exists that the study medication or the injection procedure caused the AE.
- *Not Suspected*: A reasonable possibility does not exist that the study medication and/or the injection procedure caused the AE.

A suspected adverse reaction is any AE for which there is a reasonable possibility that the study medication caused the AE. "Reasonable possibility" means there is evidence to suggest a causal relationship between the study medication and the AE. Types of evidence that would suggest a causal relationship between the study medication and the AE include the following:

- Follow a reasonable temporal sequence following the time of administration of the dose of study medication or injection procedure
- Resolve or improve after discontinuation of study medication or injection procedure
- Reappear or worsen when dosing with study medication or injection procedure is resumed

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- Are known to be a response to the study medication or injection procedure based upon pre-clinical data or prior clinical data
- Cannot be explained by other treatment(s) being administered
- Cannot be explained by the patient’s disease state or clinical condition

12.2. Serious Adverse Events (SAEs)

An AE is considered serious if it results in any of the following outcomes:

- Death; including all deaths, even those that appear to be completely unrelated to study medication
- A life-threatening AE;
 Note: An AE is considered “life-threatening” if, in the view of the Investigator, its occurrence places the subject at immediate risk of death at the time of the event. It does not include an AE that had it occurred in a more severe form, might have caused death.

- Inpatient hospitalization or prolongation of existing hospitalization;

Notes: The term “inpatient hospitalization” refers to any inpatient admission for longer than 24 hours. For chronic or long-term inpatients, inpatient admission includes transfer within the hospital to an acute/intensive care inpatient unit. Inpatient hospitalization does not include: emergency room visits or other visits if less than 24 hours; outpatient/same-day/ambulatory procedures; observation/short stay units; rehabilitation facilities; hospice facilities; or nursing homes.

The term “prolongation of existing hospitalization” refers to any extension of an inpatient hospitalization beyond the stay anticipated or required for the reason for the initial admission as determined by the Investigator or treating physician.

- A persistent or significant incapacity leading to substantial disruption of the ability to conduct normal life functions;

Note: A serious AE specifically related to visual threat (one that would require either surgical or medical intervention to prevent permanent loss of vision, or an AE that would cause substantial loss of vision) would be interpreted as any impairment to the subject’s eyesight (e.g., hemorrhage, retinal detachment, central corneal ulcer or damage to the optic nerve).

- A congenital anomaly/birth defect.
- An important medical event that may not result in death, are not life-threatening or do not require hospitalization may be considered SAEs when, based upon appropriate medical judgment, they may jeopardize the subject and may require medical or surgical intervention to prevent one of the outcomes listed in this definition.

12.2.1. Expectedness

Determination of the expectedness of an SAE should be based upon existing safety information about the study medication and injection procedure using these explanations:

- *Unexpected*: An SAE that is not listed in the Investigator's Brochure (IB)/ Summary of Product Characteristics (SmPC)/Prescribing Information/references supporting the protocol) or is not listed at the specificity or severity that has been observed.
- *Expected*: An SAE that is listed in the IB/SmPC/Prescribing Information/references supporting the protocol at the specificity and severity that has been observed.
- *Not applicable*: An SAE unrelated to the study medication.

SAEs that are mentioned in the Investigator's Brochure, Summary of Product Characteristics, or Prescribing Information/references supporting the protocol as occurring with a class of drugs or as anticipated from the pharmacological properties of the drug but are not specifically mentioned as occurring with the particular drug under investigation are to be considered unexpected.

The Investigator should initially classify the expectedness of an SAE, but the classification is subject to the Medical Monitor's determination. The Sponsor will make the final determination. All SAEs and their outcomes must be reported to the study Sponsor and its designee, and the IRB/IEC as required by the IRB/IEC, federal, state or local regulations and governing health authorities and recorded on the appropriate subject case report form and data collection tool.

12.3. Adverse events as a function of time and of number of injections

The previous subsections describe the analysis of adverse events for each of the three study groups as a whole. In addition, exploratory analyses, some tabular and some graphical, will summarize the adverse events as a function of time and of the number of injections for each of the three study groups. For example, overall adverse events and for events that occur frequently, Kaplan-Meier plots will show time to event in terms of weeks as the x-axis and injection to event where the x-axis is number of injections.

13. Subgroup Analysis

13.1. Subgroups based on baseline variables

All primary, secondary, and exploratory efficacy measures will be summarized overall and for the following subgroups, at relevant time points:

- Lesion subtype (predominantly classic, minimally classic, or occult with no classic);
- Baseline BCVA (25 to 38 letters, less than or equal to 54 letters, greater than 54 letters, and 73 or more letters);
- Baseline lens status (phakic or pseudophakic/aphakic);
- Baseline lesion size (≥ 4 disc areas or < 4 disc areas);
- Age group (50-60, 61-70, 71-80, or >80);
- Geographic region (i.e., US, EU, APAC, LATAM);

Because the number of subjects at each site is likely to be small, analyses will not be performed by study site.

14. Outline of operational details

This section sketches operational plans for the trial. Further documents will describe the operations more fully.

14.1. Database lock

The study will have two database locks: one each for the Week 36 and Week 96 data. The Week 36 database lock will include all data through Week 36; the Week 96 database lock will add data from Week 36 through Week 96. Any change to the clinical databases after each database lock will require written authorization, with explanation, by Kanghong.

Prior to each database lock, the study team will perform a masked data review and validation of the project databases. For the Week 36 lock, a named Sponsor vendor, separate from the day to day operations of the study will perform the analysis. All staff will be firewalled from post-Week 36 data by treatment group until final database lock. The Contract Research Organization (CRO) responsible for the day to day study operations and data collection will be firewalled from the unmasked Week 36 data until final database lock at Week 96. A study-specific procedure will be established to describe in detail the procedures that will be used to protect the integrity of the study from baseline

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to Week 36 and through the database lock at Week 96.

In addition, from the time of database lock, an audit trail will be maintained of all versions of the analysis data sets that may result from refinements of the algorithms for derived variables in the course of the analysis.

Should an additional lock be added, the Data Management Plan will describe the process in detail.

14.2. Authorization for unmasking

After each database lock and upon receipt of written authorization from Kanghong, the named Sponsor vendor, separate from the day to day operations of the study and responsible for analyzing the data, will receive the actual treatment codes directly from the group maintaining the treatment codes and then merge the codes to the database.

14.3. Changes from the analysis in protocol

Changes to the analyses described in this plan will be fully documented in a revised version of the plan prior to locking the study database and conducting the primary outcome analysis at Week 36. Changes made after locking the study database will be described in the clinical study report (CSR). Similarly, any change to the analyses described herein that occur between Week 36 and Week 96 will be fully documented in a revised SAP. Changes made after locking the database at Week 96 will be described in the CSR.

14.4. Archiving

After finalization of the analysis at each data lock, the following will be archived:

- All versions of the Statistical Analysis Plan
- All computer code used in creating the analyses data sets, in generating the tables and figures, and in analyzing the data
- Tables, listings, and figures as included in the clinical study report

14.5. Hardware, software, and data management

A separate technical document will describe the hardware and software to be used for this study as well as the approach to data management. Only validated software will be used.

14.6. Data quality assurance

During the course of the study the Sponsor or designee will make routine site visits to review protocol compliance, assess study drug accountability, and ensure the study is being conducted according to the relevant regulatory requirements.

The review of each subject's medical records will be performed in a manner that adequately maintains the subject's confidentiality. Further details of the study monitoring will be outlined in a separate document that describes the monitoring plan.

Regulatory authorities of domestic and foreign agencies, the Sponsor, and CRO Quality Assurance and or its designees may carry out on-site inspections or audits that may include source data checks. Therefore, direct access to the original source data will be required for inspections, audits, or both. All inspections and audits will be carried out giving consideration to data protection as well as subject confidentiality to the extent that local, state, and U.S. or other relevant national laws apply.

14.7. Study monitors

Study monitors will verify and validate the CRFs against the Investigator's source documents to ensure accurate and reliable data collection. Collected data will be entered into a computer database and subjected to electronic and manual quality assurance procedures.

15. References

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4. Sanchez MM, Chen X. Choosing the analysis population in non-inferiority studies: per protocol or intent-to-treat. *Statistics in Medicine* 2006; 25:1169–1181.
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Appendix 1. Schedule of Visits and Procedures

Study Procedure	Screening Visit 1 ^{1,5}	Day 1 Visit 2	Week 4, Visit 3 ^{2,5} Week 8, Visit 4 ^{2,5} Week 12, Visit 5 ^{2,5} Week 16, Visit 6 ² Week 20, Visit 7 ² Week 24, Visit 8 ^{2,5} Week 28, Visit 9 ² Week 32, Visit 10 ²	Week 36 Visit 11 ^{2,5}	Week 40 Visit 12 ² Week 44 Visit 13 ²	Week 48 Visit 14 ^{2,5}	Week 52, Visit 15 ² Week 56, Visit 16 ² Week 60, Visit 17 ² Week 64, Visit 18 ^{2,5} Week 68, Visit 19 ² Week 72, Visit 20 ² Week 76, Visit 21 ² Week 80, Visit 22 ² Week 84, Visit 23 ² Week 88, Visit 24 ² Week 92, Visit 25 ²	Week 96/ Exit Visit Visit 26 ^{2,5}
Informed Consent	X							
Demographic Data	X							
Medical and Ophthalmic History (including Concomitant medications)	X							
Collection of Blood and Urine Samples	X							
Urine Pregnancy Test ³	X							
Routine Blood and Urinalysis ⁴	X							X
Blood Sampling for PK and Immunogenicity ⁵	X		X	X		X	X	X
Review of Inclusion and Exclusion Criteria	X	X						
Interval History (Adverse Events and Concomitant Medications Review)	X	X	X	X	X	X	X	X
Manifest Refraction & BCVA by ETDRS	X	X	X	X	X	X	X	X
Slit-Lamp Biomicroscopy, both eyes	X	X	X	X	X	X	X	X
IOP Assessment, both eyes ⁶	X	X	X	X	X	X	X	X
SD-OCT, study eye (OU at key visits)	X	X	X	X	X	X	X	X
Dilated Indirect Ophthalmoscopy, both eyes	X	X	X	X	X	X	X	X
Fundus Photo and FA, both eyes	X			X				X
Fundus Photo, study eye ⁷			X ⁷			X ⁷	X ⁷	
Specular Microscopy ⁸	X			X				X
Registration of visit in IxR System	X	X	X	X	X	X	X	X
Randomization of Subjects		X						
IVT Injection of Study Medication or Sham Procedure ^{9,10}		X	X	X	X	X	X ¹⁰	
Posterior Segment Safety Evaluation post IVT injection of Study Medication or Sham Procedure		X	X	X	X	X	X	X
Visit Confirmation in the IxR system		X	X	X	X	X	X	X
NEI-VFQ-25		X		X		X		X
Study Exit								X

Abbreviations: AE=adverse event; BCVA=best corrected visual acuity; ETDRS= Early Treatment of Diabetic Retinopathy Study; FA=fluorescein angiography; HIPAA=Health Information Portability and Accountability Act; IOP=intraocular pressure; IVT=intravitreal; OCT=optical coherence tomography; VFQ=Visual Function Questionnaire.

1. ≤ 14 days prior to first dose of study medication.

2. ±7 days

3. Pregnancy testing only in females of childbearing potential.

4. For routine analysis of clinical chemistry, hematology, and urinalysis

5. PK and Immunogenicity testing in a subgroup of subjects where feasible.

6. Posterior segment evaluation and IOP measurement occur before injection, in the order of procedures listed, and after study treatment injection, as appropriate to minimize any risk and appropriately capture any injection related events (routine practice, should have both eyes).

7. Fundus photos, study eye only at Visit 8, Week 24, Visit 14, Week 48 and Visit 20, Week 72

8. Specular microscopy may be done only at specific sites, in a subgroup of subjects for both eyes

9. Dosing will be masked to all but the delegated unmasked investigator who administers the drug or sham procedures according to randomization and PRN criteria.

10. Evaluation for Criteria based PRN dosing regimen with maximum 4 clinic visits (approx. 16-week interval) begins at Week 52 through Week 92 (end of study treatment)

Appendix 2. Conventions

The programming conventions are described in an addendum that accompanies this SAP.

Appendix 3. Analysis of Immunogenicity

The immunogenicity data will be analyzed using descriptive methods.

Appendix 4. Analysis of the Primary and Secondary Outcomes

This Addendum describes the plans for the primary and secondary outcomes. It contains the crux of the planned SAS code and outlines each of the analyses.

Note that the SAS code presented here is considered to be pseudo code and will be finalized prior to the database lock.

Appendix 4. 1. Primary efficacy outcome:

This section outlines the approaches planned to analyze the data for the primary efficacy outcome. The primary analysis will use a mixed model for repeated measure (MMRM) with adaptive trimming of the measurements missing at Week 36 as outlined in Permutt and Li [8].

The primary efficacy analysis is based on the difference between the mean changes from baseline at Week 36 in each conbercept dose compared to the aflibercept dose estimated using an MMRM approach to trimmed data. Separate MMRM analysis will be performed comparing each conbercept dose to aflibercept. Each hypothesis of non-inferiority will be tested using a 1-sided Type I error rate of 0.0125. Trimming the data involves removing the subjects with the worst outcomes, that is, those with the worst change from baseline to Week 36 in letters gained. All drop-outs are trimmed. That makes the implicit assumption that drop-outs are among the worst performers. We recognize that a few may drop out because of reasons other than worsening vision or failure to improve; however, trimming them will have at most a trivial impact on the

results; sensitivity analyses will explore the effect of trimming them.

For each of the two comparisons, the percentage of dropouts is calculated in each of the two arms and the larger one is used for trimming. The same percentage of subjects is trimmed from both the arms that are being compared. Specifically, suppose p_a , p_8 , and p_{12} represent the proportion of drop-outs in the aflibercept, the conbercept q8 arm, and the conbercept q12 arm, respectively. Then, in comparing the conbercept q8 arm to aflibercept, the proportion of observations to be trimmed will be the $\max\{p_a, p_8\}$ in each arm. If, say, $p_a > p_8$, then all drop-outs from the aflibercept arm will be trimmed and a proportion p_a of the conbercept q8 will be trimmed; that proportion will consist of all drop-outs plus the number of those who completed the study necessary to have a proportion p_a trimmed. The completed subjects trimmed will be those who have had the worst change in vision from baseline to Week 36. A similar procedure will be performed for the comparison of conbercept q12 to aflibercept. Note that in the case where $p_{12} > p_a > p_8$, the number of trimmed subjects in the aflibercept arm will be different in the two comparisons.

To test each hypothesis, a reference distribution of treatment effect difference between conbercept and aflibercept will be constructed by permuting the original data and trimming the subjects with worst outcome. We propose using 5000 permutations to construct the reference distribution. Subjects who have dropped out and do not have a Week 36 measurement will be removed each time from the permuted data, making sure that the same percentage of data is excluded from both the conbercept arm and aflibercept arm as described above. Because we are using adaptive trimming, the percentage of subjects trimmed in each permuted data will vary slightly. The reference distribution will be used to calculate the p-value.

The 97.5% confidence interval (97.5% CI) for the mean difference between two drug groups will be constructed following one of the approaches outlined in paper by Permutt and Li [8]. Specifically, from the permutation distribution based on 5000 permutations, we will first calculate the 1.25 and 98.75 percentiles; we will then add the observed trimmed

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mean difference to each in order to estimate the lower bound and upper bounds of the 97.5% CI for the difference between each conbercept arm and the aflibercept arm. Note that as Permutt and Li described, this confidence interval is slightly conservative. This confidence interval will be adjusted based on the number of IDMC meetings as described in Section 2.4.

In summary, we will use the following approach for each of the two analyses separately:

- 1) Determine the percentage of subjects missing Week 36 measurement in the conbercept dose and the aflibercept dose.
- 2) Choose the larger of the two percentages (say x%).
- 3) Sort the subjects in each arm from worst to best change in visual acuity from baseline at Week 36 and remove the worst x% of the subjects from each of the two arms where a drop-out is considered one of the worst outcomes.
- 4) Use PROC MIXED to obtain an estimate of the difference between the conbercept dose and the aflibercept dose. Should the mixed model fail to converge an analysis of covariance model will be used.
- 5) Use PROC SURVEYSELECT to choose a random sample of subjects from the original data to form a new conbercept arm with the same sample size as the actual conbercept arm. The remaining subjects will form the aflibercept arm.
- 6) Repeat steps 1 to 5 to obtain one possible treatment effect.
- 7) Repeat steps 5 and 6 until 5000 possible treatment effects are obtained.
- 8) Calculate the p-value as the proportion of the times the observed treatment effect is greater than or equal to the estimated treatment effect.
- 9) Construct the 97.5% CI for the mean difference of treatment effect between two drug arms based on 1.25 percentile and 98.75 percentile of permuted distribution and observed mean difference between the two drug arms. The percentiles will be adjusted for the number of IDMC

meetings as described in Section 2.4.

Appendix 4. 2. SAS code for analyzing each hypothesis

The pseudo SAS code we plan to use is presented below. The first step is to trim the worst change from baseline at Week 36 from each group making sure that the same percentage of subjects is trimmed from both. The trimmed data from both arms are combined and used to obtain an estimate of the treatment effect of the conbercept dose compared to the aflibercept dose. This code may be modified before the final database lock.

Exhibit 4 1. SAS code to compute estimate treatment effect

```
proc mixed data=TRIMMED;
  class TRTP (ref=AFLIB) AVISIT USUBJID;
  model CHG_BCVA = BASE TRTP AVISIT AVISIT * TRTP
    / ddfm=kr residual solution;
  repeated AVISIT / type=UN subject=USUBJID;
  estimate 'primary_efficacy1' TRTP 1 -1
    AVISIT*TRTP 0 0 0 0 0 0 0 0 1 0 0 0 0 0 0 0 0 -1;
  ods output estimate=SI_estimates ;
run;
```

The next step is to construct the reference distribution of treatment effects. A permuted dataset is obtained from the original data by random sampling without replacement and the same sample size as the conbercept arm (say N_{c8}) and the remaining dataset is a sample for the aflibercept arm. The code for randomly choosing N_{c8} subjects from all the subjects is outlined in **Exhibit 4 2**.

Exhibit 4 2. SAS code for selecting subjects to generate one possible conbercept arm

```
proc surveyselect data=ORIGINAL
  out= CONBERCEPT method=SRS samplesize= $N_{c8}$  seed=128258;
run;
```

The subjects in the dataset CONBERCEPT are excluded from the dataset ORIGINAL to obtain the subjects in the corresponding aflibercept arm. The percentage of subjects with missing Week 36 measurement in each of these two

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datasets is determined and the largest percentage is used for trimming the worst outcomes from both. The combined trimmed dataset is one possible permutation that will be used for constructing the reference distribution. Changing the seed will yield different samples. Each seed will be chosen by the following formula for the k'th permuted and trimmed dataset: $128258 + (k-1) \times 8729$.

By repeating the process outlined above, a total of 5000 permuted and trimmed datasets will be generated. Each of these datasets is analyzed using the code outlined in **Exhibit 4 1** to obtain a treatment effect. The number of times the treatment effect is greater than or equal to the estimated treatment effect divided by 5000 is the one-sided p-value. This p-value will be compared to the Type I error rate of 0.0125 to determine significance. The two-sided 97.5% CI will be constructed based on the 1.25 and 98.75 percentiles of the permutation distribution; non-inferiority will be assessed by comparing the lower bound of 97.5% CI with the pre-specified non-inferiority margin with an additional adjustment for the number of IDMC meetings as described in Section 2.4.

Appendix 4. 3. Secondary efficacy outcomes:

Table 3 shows the secondary outcomes, their non-inferiority margins, and a brief summary of the plans for analyses. For two of the secondary endpoints, change in retinal thickness from baseline to week 36 and change in BCVA from baseline to week 96 additional information regarding the choice of the NIM is provided below.

- Change in retinal thickness from baseline to week 36 – Based on feedback from FDA, Kanghong has proposed a non-inferiority margin of 25µm, as this represents approximately 10-15% of normal central retinal thickness. Retinal edema and thickness are widely used clinically as markers of disease activity. It is recognized, however, that the correlation between retinal thickness and BCVA had not been established, and this will limit the utility, analysis, and any potential labeling.
- Change in BCVA from baseline to week 96 – Based on feedback from the agency, Kanghong has proposed to use a NI margin of -4.0 ETDRS letters, as this margin is considered clinically relevant, and was also used to compare Aflibercept

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to Ranibizumab at week 96 in the VIEW 1&2 studies (Schmidt-Erfurth et al, 2014). Patients in the Aflibercept 2q8 arm gained on average 7.6 ETDRS letters at week 96 compared to baseline. It is fully recognized that the week 96 data from the VIEW studies includes the capped-PRN period and is not well controlled. These limitations impact the utility of the analysis as well as any potential labeling.

For the secondary outcomes evaluating change from baseline in BCVA and retinal thickness, the SAS code is analogous to the coder for the primary efficacy outcome. For the other secondary outcomes evaluating the proportion who maintained vision from baseline (i.e., losing <15 BCVA letters from baseline) and the proportion who gained at least 15 letters by week 36, PROC FREQ will be used to estimate a chi-squared test as indicated below. The significance levels of these analyses will be adjusted using the method outlined in Section 2.4.

Exhibit 4 6. SAS code to obtain the overall estimate and standard error

```
proc freq data=&PAR. &POP. order=data;
  tables &TRT.*&CRIT./ riskdiff(noninf margin=&NIM. Method=fm) alpha=0.0025;
  where &TRT.N in (1,3);
ods output PdiffNoninf=diffnoninf_low;
run;
```

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Table 3. Secondary efficacy outcomes

Outcome	Statistic	NI margin	Assumptions and Analysis	Power
Maintenance of vision from baseline to Week 36	Proportion maintaining vision at Week 36 (i.e., losing <15 BCVA letters from baseline to Week 36)	-0.07	Assumptions: 85% of all participants have a Week 36 measure; 95% of participants in all groups maintain their baseline BCVA. Analysis: Chi-square test. All subjects with missing values will be treated as nonresponders.	>95%
Gaining at least 15 letters from baseline to Week 36	Proportion gaining at least 15 letters by Week 36	-0.10	Assumptions: 85% of all participants have a Week 36 measure and 35% of participants in all groups gain 15 letters. Analysis: Chi-square. All subjects with missing values will be treated as nonresponders.	50%
Change in retinal thickness from baseline to Week 36	Mean change in retinal thickness (μm) by Week 36	25μm	Assumptions: 80% of all participants have measurements at Week 36; standard deviation of change is 110μm. Analysis: Longitudinal model (MMRM) analogous to the model planned for the primary non-inferiority outcome.	to be determined
Maintenance of vision from baseline to Week 48	Proportion maintaining vision at Week 48 (i.e., losing <15 BCVA letters from baseline to Week 48)	-0.07	Assumptions: 85% of all participants have a Week 48 measure; 95% of participants in all groups maintain their baseline BCVA. Analysis: Chi-square test. All subjects with missing values will be treated as nonresponders.	>95%
Change in BCVA from baseline to Week 96	Mean change in BCVA from baseline to Week 96	-4.0	Assumptions: 80% of all participants have a Week 96 measure; standard deviation of BCVA is 16 letters. Analysis: Longitudinal model (MMRM) analogous to that planned for the primary non-inferiority outcome.	80%
NI=non-inferiority				

Appendix 4. 4. Exploratory efficacy outcomes:

Each exploratory outcome and the analysis approach for each outcome are described below.

To evaluate differences between the conbercept doses and aflibercept with respect to the following outcome measures in the study eye:

- Mean change from baseline in ETDRS BCVA letter score at all study visits, except for Week 96 will be presented both descriptively and with a MMRM model as described in the addendum to this SAP.
- Proportion of subjects maintaining vision (i.e., losing <15 ETDRS BCVA letters) at each visit, except for Week 36 and Week 48 will be presented using a descriptive analysis. A chi-square test will be used to compare groups.
- Proportion of subjects with 20/40 vision equivalent (74 to 78 BCVA letters) or better at Week 36, 48, 96, and each study visit will be presented using a descriptive analysis. A chi-square test will be used to compare groups.
- Proportion of subjects losing 5, 10, and 15 letters or more at every visit from Week 36 to Week 96 will be presented using a descriptive analysis. A chi-square test will be used to compare groups.
- Proportion of subjects gaining 5, 10, and 15 letters or more at every visit from Week 36 to Week 96 (except for proportion of subjects gaining 15 letters or more at Week 36) will be presented descriptively. A chi-square test will be used to compare groups.
- Mean and/or mean change from baseline in all available continuous anatomical parameters obtained from fluorescein angiography (FA), for example, the choroidal neovascularization (CNV) area, at Week 36 and Week 96 will be analyzed descriptively and with MMRM.
- Categorical anatomical parameters obtained from FA, for example, the proportion of patients with leakage at Week 36 and Week 96 will be presented descriptively with chi-square analyses used to compare groups.
- Mean change from baseline in anatomic measures, as measured by OCT, including total macular volume, persistent fluid, pigment epithelium detachment (PED) height (or volume) as assessed by SD-OCT at Week 36, 48, 96, and each study visit and central retinal thickness at Week 48 and 96 and each study visit will be presented descriptively and with a MMRM as described in the addendum to this SAP.
- Categorical anatomical parameters based on OCT, for example, the proportion of patients with dry retina at each Week 36, 48, 96, and each study visit will be presented descriptively with chi-square analyses used to compare groups.
- Mean change from baseline in visual function as assessed by the National Eye Institute Visual Function Questionnaire (NEI-VFQ)-25 at Week 36, 48, and 96 will be presented descriptively and with a MMRM as described in the addendum to this SAP.
- Analysis and distribution of re-treatment in the PRN phase from Week 52 to Week 92 will be presented descriptively based on parameters such as number of injections. If applicable, two-sample testing will be used.
- Analysis and distribution of re-treatment criteria status from Week 52 to Week 92 will be presented descriptively. If applicable, two-sample testing will be used.